



SVOZ

0.5% w/v OPHTHALMIC SOLUTION

PRODUCT NAME

SVOZ (0.5% OPHTHALMIC SOLUTION)

NAME AND STRENGTH OF ACTIVE SUBSTANCE

1 mL of solution contains: 5.45 mg moxifloxacin hydrochloride (equivalent to 5 mg moxifloxacin)

PRODUCT DESCRIPTION

Clear greenish-yellow solution.

PHARMACODYNAMICS

Pharmacotherapeutic group: ophthalmologicals, anti-infectives, other anti-infectives

Mechanism of Action

Moxifloxacin, a fourth-generation fluoroquinolone, inhibits the DNA gyrase and topoisomerase IV required for bacterial DNA replication, repair, and recombination. The C8-methoxy moiety of moxifloxacin lessens the selection of resistant mutants of Gram-positive bacteria compared to the C8-H moiety found in older fluoroquinolones. Moxifloxacin's bulky C-7 substituent group interferes with the quinolone efflux pump mechanism of bacteria. Moxifloxacin is often bactericidal at concentrations equal to or slightly greater than inhibitory concentrations.

Microbiological Susceptibility

Fluoroquinolones, including moxifloxacin, differ in chemical structure and mode of action from β -lactam antibiotics, macrolides and aminoglycosides. Therefore, organisms resistant to these drugs may be susceptible to moxifloxacin. In vitro resistance to moxifloxacin develops slowly via multiple-step mutations and occurs at a general frequency between 10⁻⁹ to 10⁻¹¹ for Gram-positive bacteria.

Moxifloxacin has been shown to be active against most strains of the microorganisms as tabulated in **Figure 1** below, both in vitro and in clinical infections.

Gram-Positive Bacteria:
■ <i>Corynebacterium</i> species* ■ <i>Microbacterium</i> species ■ <i>Micrococcus luteus</i>* [including erythromycin, gentamicin, tetracycline, and/or trimethoprim resistant strains] ■ <i>Staphylococcus aureus</i> [including methicillin, erythromycin, gentamicin, ofloxacin, tetracycline, and/or trimethoprim resistant strains] ■ <i>Staphylococcus epidermidis</i> [including methicillin, erythromycin, gentamicin, ofloxacin, tetracycline, and/or trimethoprim resistant strains] ■ <i>Staphylococcus haemolyticus</i> [including methicillin, erythromycin, gentamicin, ofloxacin, tetracycline, and/or trimethoprim resistant strains] ■ <i>Staphylococcus hominis</i>* [including methicillin, erythromycin, gentamicin, ofloxacin, tetracycline, and/or trimethoprim resistant strains] ■ <i>Staphylococcus warneri</i>* [including erythromycin resistant strains] ■ <i>Streptococcus mitis</i>* [including penicillin, erythromycin, tetracycline, and/or trimethoprim resistant strains] ■ <i>Streptococcus pneumoniae</i> [including penicillin, erythromycin, gentamicin, tetracycline, and/or trimethoprim resistant strains] ■ <i>Streptococcus viridans</i> [including penicillin, erythromycin, tetracycline, and/or trimethoprim resistant strains]
Gram-Negative Bacteria:
■ <i>Acinetobacter</i> species ■ <i>Haemophilus "alconae"</i> [including ampicillin resistant strains] ■ <i>Haemophilus influenzae</i> [including ampicillin resistant strains] ■ <i>Klebsiella pneumoniae</i>* ■ <i>Moraxella catarrhalis</i>* ■ <i>Pseudomonas aeruginosa</i>*
Other Microorganisms:
■ <i>Chlamydia trachomatis</i>

Note: * indicates efficacy for this organism was studied in fewer than 10 infections

Moxifloxacin has been shown to be active in vitro against most strains of the organisms as listed in **Figure 2** below. However, the clinical significance of these data is unknown.

Gram-Positive Bacteria:	
<i>Arthrobacter</i> species <i>Bacillus cereus</i> <i>Bacillus thuringiensis</i> <i>Corynebacterium accolens</i> <i>Corynebacterium amycolatum</i> <i>Corynebacterium bovis</i> <i>Corynebacterium macginleyi</i>	<i>Staphylococcus caprae</i> <i>Staphylococcus cohnii</i> <i>Staphylococcus lugdunensis</i> <i>Staphylococcus pasteurii</i> <i>Staphylococcus saprophyticus</i> <i>Staphylococcus sciuri</i> <i>Streptococcus agalactiae</i>

Gram-Positive Bacteria:	
<i>Corynebacterium propinquum</i> <i>Corynebacterium pseudodiphtheriticum</i> <i>Enterococcus faecalis</i> <i>Exiguobacterium</i> species <i>Kocuria kristinae</i> <i>Kocuria "lindaee"</i> <i>Kocuria rhizophila</i> <i>Listeria monocytogenes</i> <i>Microbacterium "harmaniae"</i> <i>Microbacterium "ottidii"</i> <i>Rathia mucilaginosus</i> <i>Staphylococcus arlettae</i> <i>Staphylococcus capitis</i>	<i>Streptococcus "conjunctivae"</i> <i>Streptococcus cristatus</i> <i>Streptococcus dysgalactiae</i> <i>Streptococcus mitis</i> <i>Streptococcus Group C, G and F</i> <i>Streptococcus "ocularis"</i> <i>Streptococcus oralis</i> <i>Streptococcus parasanguinis</i> <i>Streptococcus pyogenes</i> <i>Streptococcus salivarius</i> <i>Streptococcus sanguis</i> <i>Streptococcus "schlechlii"</i>
Gram-Negative Bacteria:	
<i>Achromobacter xylosoxidans</i> <i>Acinetobacter baumannii</i> <i>Acinetobacter calcoaceticus</i> <i>Acinetobacter johnsonii</i> <i>Acinetobacter junii</i> <i>Acinetobacter lwoffii</i> <i>Acinetobacter "mumbaiensis"</i> <i>Acinetobacter schindlerii</i> <i>Acinetobacter ursingii</i> <i>Aeromonas caviae</i> <i>Chryseobacterium indologenes</i> <i>Chryseobacterium</i> species <i>Citrobacter freundii</i> <i>Citrobacter koseri</i> <i>Enterobacter aerogenes</i>	<i>Enterobacter cloacae</i> <i>Enterobacter hormaechei</i> <i>Escherichia coli</i> <i>Klebsiella oxytoca</i> <i>Moraxella osloensis</i> <i>Morganella morganii</i> <i>Neisseria gonorrhoeae</i> <i>Pantoea agglomerans</i> <i>Proteus mirabilis</i> <i>Proteus vulgaris</i> <i>Pseudomonas oryzae</i> <i>Pseudomonas stutzeri</i> <i>Serratia liquefaciens</i> <i>Serratia marcescens</i> <i>Stenotrophomonas maltophilia</i>
Anaerobic Microorganisms:	
<i>Clostridium perfringens</i> <i>Fusobacterium</i> species <i>Porphyromonas</i> species	<i>Prevotella</i> species <i>Propionibacterium acnes</i>
Other Organisms:	
<i>Atypical Mycobacterium</i> <i>Chlamydia pneumoniae</i> <i>Legionella pneumophila</i>	<i>Mycobacterium avium</i> <i>Mycobacterium marinum</i> <i>Mycoplasma pneumoniae</i>

PHARMACOKINETICS

Following topical ocular administration, moxifloxacin was absorbed into the systemic circulation. The mean steady-state of maximum plasma concentration (C_{max} = 2.7 ng/mL) and plasma concentration-time curve (AUC = 41.9 ng.hr/mL) are approximately 1,600 and 1,200 times lower than the mean C_{max} and AUC reported after therapeutic 400 mg oral doses of moxifloxacin. The plasma half-life of moxifloxacin was estimated to be 13 hours.

INDICATIONS

- SVOZ (0.5%w/v OPHTHALMIC SOLUTION) is indicated for the treatment of patients 1 year of age and older with bacterial conjunctivitis caused by susceptible strains of the following organisms:

Gram-positive bacteria:

Corynebacterium species†

Microbacterium species

Micrococcus luteus† [including erythromycin, gentamicin, tetracycline, and/or trimethoprim resistant strains]

Staphylococcus aureus [including methicillin, erythromycin, gentamicin, ofloxacin, tetracycline and/or trimethoprim resistant strains]

Staphylococcus epidermidis [including methicillin, erythromycin, gentamicin, ofloxacin, tetracycline and/or trimethoprim resistant strains]

Staphylococcus haemolyticus [including methicillin, erythromycin, gentamicin, ofloxacin, tetracycline and/or trimethoprim resistant strains]

Staphylococcus hominis† [including methicillin, erythromycin, gentamicin, ofloxacin, tetracycline and/or trimethoprim resistant strains]

Staphylococcus warneri† [including erythromycin resistant strains]

Streptococcus mitis† [including penicillin, erythromycin, tetracycline and/or trimethoprim resistant strains]

Streptococcus pneumoniae [including penicillin, erythromycin, gentamicin, tetracycline and/or trimethoprim resistant strains]

Streptococcus viridans [including penicillin, erythromycin, tetracycline and/or trimethoprim resistant strains]

Gram-negative bacteria:

Acinetobacter species

Haemophilus "alconae" [including ampicillin resistant strains]

Haemophilus influenzae [including ampicillin resistant strains]

Klebsiella pneumoniae†

Moraxella catarrhalis†

*Pseudomonas aeruginosa**

Other microorganisms:

Chlamydia trachomatis

Actual Size 100 %

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[Note: † indicates efficacy for this organism was studied in fewer than 10 infections]
■ Preoperative and postoperative sterilization (when prophylactic antibiotic treatment is required)

RECOMMENDED DOSAGE

- Instill one drop in the affected eye 3 times a day for 7 days

For Preoperative and Postoperative Sterilization

- Usually, instill one drop in the affected eye(s) 5 times per day before operation, and 3 times per day after operation

ROUTE OF ADMINISTRATION

For ocular use only. Not for injection. SVOZ ophthalmic solution should not be injected subconjunctivally or introduced directly into the anterior chamber of the eye. To prevent contamination of the applicator tip and solution, care must be taken not to touch the eyelids, fingers or surrounding areas with the dropper tip of the bottle.

CONTRAINDICATIONS

Hypersensitivity to the active substance, to other quinolones, or to any of the excipients of this product.

WARNINGS AND PRECAUTIONS

Hypersensitivity Reactions

In patients receiving systemically administered quinolones, serious and occasionally fatal hypersensitivity (anaphylactic) reactions have been reported, some following the first dose. Some reactions were accompanied by cardiovascular collapse, loss of consciousness, angioedema (including laryngeal, pharyngeal or facial oedema), airway obstruction, dyspnoea, urticaria, and itching. If an allergic reaction occurs, discontinue use of the drug. Serious acute hypersensitivity reactions may require immediate emergency treatment.

Oxygen and airway management should be administered where clinically indicated.

Overgrowth of Non-Susceptible Microorganisms

As with other anti-infectives, prolonged use may result in overgrowth of non-susceptible organisms, including fungi. If superinfection occurs, discontinue use and institute alternative therapy.

Tendon Inflammation

Tendon inflammation and rupture may occur with systemic fluoroquinolone therapy including moxifloxacin, particularly in elderly patients and those treated concurrently with corticosteroids. Therefore, caution should be exercised and treatment should be discontinued at the first sign of tendon inflammation.

Exacerbation of Myasthenia Gravis

Fluoroquinolones have neuromuscular blocking activity and may exacerbate muscle weakness in person with myasthenia gravis. Post marketing serious adverse events, including deaths and requirement for ventilator support have been associated with fluoroquinolones use in persons with myasthenia gravis. Avoid fluoroquinolones in patients with known history of myasthenia gravis.

Risk of Cross-Contamination

Patients should be advised not to wear contact lenses if they have signs and symptoms of a bacterial ocular infection.

Fluoroquinolone-Resistant *Neisseria gonorrhoeae*

SVOZ (0.5%w/v OPHTHALMIC SOLUTION) should not be used for the prophylaxis or empiric treatment of gonococcal conjunctivitis, including gonococcal ophthalmia neonatorum, because of the prevalence of fluoroquinolone-resistant *Neisseria gonorrhoeae*.

Patients with eye infections caused by *Neisseria gonorrhoeae* should receive appropriate systemic treatment.

Paediatric Population

Data are very limited to establish efficacy and safety of moxifloxacin eye drops solution in the treatment of conjunctivitis in neonates. Therefore, use of this medicinal product to treat conjunctivitis in neonates is not recommended.

Neonates with ophthalmia neonatorum should receive appropriate treatment for their condition (e.g. systemic treatment in cases caused by *Chlamydia trachomatis* or *Neisseria gonorrhoeae*). The medicinal product is not recommended for the treatment of *Chlamydia trachomatis* in patients less than 2 years of age as it has not been evaluated in such patients. Patients older than 2 years of age with eye infections caused by *Chlamydia trachomatis* should receive appropriate systemic treatment.

EFFECTS ON THE ABILITY TO DRIVE AND USE MACHINES

SVOZ (0.5%w/v OPHTHALMIC SOLUTION) has no or negligible influence on the ability to drive and use machines, however, as with any eye drops, temporary blurred vision or other visual disturbances may affect the ability to drive or use machines. If blurred vision occurs at instillation, the patient should wait until their vision clears before driving or using machinery.

INTERACTIONS WITH OTHER MEDICAMENTS

No specific interaction studies have been performed. Given the low systemic concentration of moxifloxacin following topical ocular administration of the medicinal product (See *Pharmacokinetics*), drug interactions are unlikely to occur.

PREGNANCY AND LACTATION

Pregnancy

There are no or limited amount of data from the use of SVOZ (0.5%w/v OPHTHALMIC SOLUTION) in pregnant women. However, no effects on pregnancy are anticipated since the systemic exposure to moxifloxacin is negligible. The medicinal product can be used during pregnancy.

Lactation

It is unknown whether moxifloxacin/metabolites are excreted in human milk. Animal studies have shown excretion of low levels in breast milk after oral administration of moxifloxacin. However, at therapeutic doses of SVOZ (0.5%w/v OPHTHALMIC SOLUTION) no effects on the suckling child are anticipated. The medicinal product can be used during breast-feeding.

Fertility

Studies have not been performed to evaluate the effect of ocular administration of SVOZ (0.5%w/v OPHTHALMIC SOLUTION) on fertility.

UNDESIRABLE EFFECTS

Summary of Safety Profile

The adverse reactions presented below are classified by system organ and frequency in decreasing order of the seriousness.

System Organ	Frequency	Adverse Reactions
Blood and lymphatic system disorders	Rare	: Haemoglobin decreased
Immune system disorders	Not known	: Hypersensitivity
Nervous system disorders	Uncommon Rare Not known	: Headache : Paresthesia : Dizziness
Eye disorders	Common Uncommon Rare Not known	: Eye pain, eye irritation : Punctate keratitis, dry eye, conjunctival haemorrhage, ocular hyperaemia, eye pruritus, eyelid oedema, ocular discomfort : Corneal epithelium defect, conjunctivitis, blepharitis, corneal disorder, eye swelling, conjunctival oedema, vision blurred, visual acuity reduced, asthenopia, erythema of eyelid : Endophthalmitis, ulcerative keratitis, corneal erosion, corneal abrasion, intraocular pressure increased, corneal opacity, corneal infiltrates, corneal deposits, eye allergy, keratitis, corneal oedema, photophobia, eyelid oedema, lacrimation increased, eye discharge, foreign body sensation in eyes
Cardiac disorders	Not known	: Palpitations
Respiratory, thoracic and mediastinal disorders	Rare Not known	: Nasal discomfort, pharyngolaryngeal pain, sensation of foreign body (throat) : Dyspnoea
Gastrointestinal disorders	Uncommon Rare Not known	: Dysgeusia : Vomiting : Nausea
Hepatobiliary disorders	Rare	: Alanine aminotransferase increased, gamma-glutamyltransferase increased
Skin and subcutaneous tissue disorders	Not known	: Erythema, rash, pruritus, urticaria

Post Marketing Surveillance

Additional adverse reactions identified by post marketing experience may include the following :- (seriousness is presented in no particular order; frequency cannot be estimated from available data)

System Organ	Adverse Reactions
Immune system disorders	Hypersensitivity
Nervous system disorders	Dizziness
Eye disorders	Ulcerative keratitis, keratitis, lacrimation increased, photophobia, eye discharge
Cardiac disorders	Palpitations
Respiratory, thoracic and mediastinal disorders	Dyspnoea
Gastrointestinal disorders	Nausea
Skin and subcutaneous tissue disorders	Erythema, pruritus, rash, urticaria
Musculoskeletal	Exacerbation of myasthenia gravis

OVERDOSE AND TREATMENT

The limited holding capacity of the conjunctival sac for ophthalmic products practically precludes any overdosing of the medicinal product. If a topical overdose of moxifloxacin eye drops occurs, the eye(s) may be flushed with tap water.

The total amount of moxifloxacin in a single container is too small to induce adverse effects after accidental ingestion.

STORAGE CONDITIONS

Store below 30°C.

Protect from light.

(The product should be stored below 30°C after opening)

SHELF LIFE

Refer to unit box for shelf-life.

Discard 30 days after first opening.

DOSAGE FORMS AND PACKAGING AVAILABLE

Available in 1 x 5 mL white LDPE plastic bottle with clear LDPE plastic nozzle and pale blue HDPE plastic screw cap, inserted in a paper box.

DATE OF REVISION OF PI

Date of revision : 30/01/2023

Manufacturer:

SIAM BHEASACH CO., LTD.

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Product Registration Holder, Importer:

SPG PHARMA (MALAYSIA) SDN BHD

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Actual Size 100 %

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